



Transforming education through health and well-being

A briefing note



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About this note

This note argues for integrating health and well-being into education policy and planning so that education systems promote health and well-being in ways that better enable learners to learn. Ensuring that health and well-being are core markers of education requires supportive policies and laws, safe and health-promoting environments in schools, skills-based education about health and well-being, and health and nutrition services in schools. Each of these measures is an important part of a transformative education agenda that prepares learners for the future, builds human capital, including social and emotional skills, and links with topics such as climate, food systems and global citizenship.

The note is aimed at education sector policy-makers and professionals, both in government and outside of it, as well as those from other sectors who work in the field of children's and young people's health and well-being. It summarizes recent research on the promotion of health and well-being through education globally, including impacts and the extent to which health and well-being are integrated into education sector policies and planning processes. It surveys the evidence reviews, guidelines and tools that are available to support policy-makers, planners and development partners in prioritizing and promoting health and well-being among learners and in the wider community. Many countries need support to use these tools and integrate health and well-being strategies into their education policy and planning and this note outlines the role that development partners can play in providing this support.

The note has been developed as a companion to UNESCO's *Integrating Health And Well-Being In Education Sector Planning: A Handbook*. Both projects share a common methodology: a comprehensive desk review of academic and grey literature, analysis of national education plans and commitments made at the 2022 Transforming Education Summit, expert consultations through an inter-agency group on school health and nutrition, and practical insights from pilot workshops in Malawi and the Maldives. Tools were selected through an iterative expert review process, drawing on their relevance to current education sector planning practice, their use in recent country contexts and their availability from recognized sources. Although a formal grading of evidence was not applied, findings were triangulated across multiple sources and refined through peer review and stakeholder feedback. Further methodological details are provided in the handbook.

The development process was led by UNESCO in collaboration with partners from GPE, Research Consortium for School Health and Nutrition, UNESCO Chair on Global Health and Education, UNICEF, The World Bank, WFP and WHO, and the content development involved experts from across these agencies, academia and global initiatives. We gratefully acknowledge the contributions of the inter-agency working group on school health and nutrition, government stakeholders in pilot countries and all peer reviewers who supported the process.

This note was made possible by generous financial support from the Governments of Sweden and Norway.

Key messages

- Research shows that learners who are healthy and safe stay in school longer and achieve better educational outcomes. They also have a lower risk of grade repetition, dropout and absenteeism.
- Children's and adolescents' health and well-being are essential to foundational learning and ending learning poverty, as well as being important goals in their own right.
- Countries around the world are increasingly cognizant of the connections between health, well-being and education and recognize the need to work in multisectoral ways to ensure that children and adolescents grow up healthy, productive and prepared for the future.
- In order to make this vision a reality, countries can take the following steps:
 - Build gender-equal, safe, secure, inclusive, climate-ready, health-promoting physical and social environments in schools by ensuring that all schools have, for example, water, sanitation and hygiene (WASH); safe infrastructure; adequate space for play; facilities for physical education; infrastructure for children with disabilities; measures to prevent school-related violence; and healthy food environments. School environments must also foster respect for others and promote healthy relationships.
 - Provide skills-based education on health and well-being that is integrated throughout the curriculum and in extracurricular activities, progressively building knowledge and skills across all grades while remaining adaptable to emerging health or environmental conditions. Skills-based topics include health literacy, social-emotional competencies, sexual and reproductive health, healthy lifestyles, mental health, nutrition, oral health, menstrual health and hygiene, disease prevention, physical education, safety, and risk awareness and reduction.
 - Offer comprehensive school health and nutrition services, including vaccination programmes, school meals, micronutrient supplementation, deworming, vision and hearing screening, and physical and mental health assessments with a referral system for those who need treatment or support.
- Governments need to adopt a multisectoral approach to education sector planning and policy-making, involving health and other social sectors to support learners' development. Better coordination with ministries of planning and finance is essential to ensure adequate funding of health and well-being measures in education. Policies and planning should also integrate perspectives from students and teachers and respond effectively to needs at national, subnational and school levels.
- International organizations, multi- and bilateral agencies, civil society and other actors need to intensify their dialogue with governments in order to improve coordination with a focus on the long-term sustainability of integrated health and well-being in education systems.
- These elements are best seen as part of a wider, holistic vision of transformative education: education that supports learners' gender equality, health and well-being; advances their understanding of sustainable development, climate, peace and citizenship; enhances their potential future earnings and productivity; gives them the social and emotional skills they need for their current and future well-being; and addresses the social norms and inequalities that create health disparities.

1. Introduction

Children and adolescents who are healthy and well-nourished, who engage in regular physical activity and who have access to safe learning environments learn better and benefit from a range of favourable health outcomes (Basch, 2011; Eide and Showalter, 2011; Hamad et al., 2018; Owen et al., 2018; Balaj et al., 2024). Increasingly, countries recognize the link between health and education, and most countries now have policy frameworks related to school health and nutrition or provide health services in schools (UNESCO et al., 2023). The unprecedented school closures triggered by the COVID-19 pandemic highlighted the connections between health, well-being and learning and emphasized the critical role that schools play in ensuring educational continuity and supporting children's health during the crisis (Alban Conto et al., 2020).

The 2022 Transforming Education Summit underscored the global recognition of the interconnections between health, well-being and education, with nearly two-third of countries' statements of commitment explicitly highlighting these areas as essential for educational success. Over 80 countries committed to enhanced measures to promote the physical and mental health of learners and to ensure their safety (UNESCO, 2024b). The Commitment to Action on Foundational Learning, launched at the summit and endorsed by 28 countries and by participating organizations, affirms the need to support the health, nutrition and psychosocial well-being of every teacher and child in order to ensure that all children can realize their full potential (UNICEF, 2022).

Yet, many countries struggle to articulate concrete measures to achieve these goals. Education sector plans often lack initiatives that would improve school health and well-being outcomes, and the school health community and development partners, including the United Nations, urgently need to provide support to countries to make the vision of integrated health and well-being in education a reality.

Health and well-being in education may sometimes be seen as competing for resources with interventions

focused on foundational learning. In many low- and lower-middle-income countries, large numbers of children and adolescents remain either out of school or unable to attain basic literacy and numeracy skills. Facing an urgent need to tackle this widespread learning poverty (World Bank, 2019), lower-income countries have focused on numeracy and literacy as a means to improving foundational learning (UNICEF, 2022). However, rather than being seen as a competing priority to improving numeracy and literacy, health and well-being in education needs to be understood as a fundamental part of learning and integral to supporting foundational learning through a holistic understanding of students' needs.

Intersectoral action is needed to tackle inequalities in child health (Solar et al., 2023), gender and education (GPE, 2023). If political will and finances are to be mobilized and cross-sectoral linkages made, decision-makers will need to understand the connections between health and education. Ministries of education will also need to be supported to build their technical capacities if they are to achieve these goals in practice.

This note makes the case for health and well-being to be integrated into education sector policy, planning and programmes as a high priority. Many countries have taken important steps towards providing school health and nutrition services, but a more holistic approach is needed that also includes attention to and investment in policies and systems that provide an enabling environment at national, subnational and school levels for safe, health-promoting and well-being-centred school environments. In addition, health and well-being need to be delivered through skills-based education and extracurricular activities that comprehensively address learners' needs as part of a transformative vision for education that equips learners with a range of skills and competencies they will need throughout their lives. This note summarizes the available guidance and range of tools that can be used to support this process; it also argues that education planners and policy-makers need additional support to put the tools and guidance into practice.

2. Why health and well-being in education matter

As the evidence on health, well-being and education shows, investing in the health and nutrition of school-age children and adolescents optimizes investment in education (Bundy et al., 2018).

2.1 Health and well-being lead to better learning

Health disparities in children directly and indirectly affect participation, attendance, retention and academic achievement (Basch, 2011), reinforcing inequalities by gender and other dimensions in educational outcomes. Many learners miss school or do not learn well while at school due to preventable or treatable illness and hunger (Basch, 2011; Eide and Showalter, 2011; Hamad et al., 2018; Owen et al., 2018). Learners in low- and lower-middle-income countries miss up to 500 million schooldays each year due to ill health (Bundy, 2011). Worm infections, for example, have been shown to have large effects on school participation, particularly among young children; a deworming intervention in Kenya increased school participation by up to 10 percentage points (Miguel and Kremer, 2004). Children who are bullied are twice as likely to skip school as those who are not (UNESCO, 2019a). When students feel supported emotionally and psychologically, they are more likely to attend school regularly, participate actively and achieve academic success (WHO et al., 2023). Health issues and violence disproportionately affect girls, children with disabilities and children from marginalized groups, especially those who are subjected to multiple, cumulative forms of discrimination (UNESCO et al., 2023).

Health interventions that directly address these disparities among children and adolescents allow affected students to participate more regularly in school and learn better (Box 1). Deworming programmes and micronutrient supplementation have resulted in children attaining up to 2.5 additional years of schooling in areas where helminth infections and anaemia are prevalent (Bundy, 2011). Vision screening and glasses for children with poor vision in rural China led to test score improvements of 0.16 to 0.22 standard deviations in reading and mathematics – equivalent to between 0.3 and 0.5 additional years of schooling (Glewwe et al., 2016). In a number of studies, providing nutritious school meals and supplements has been effective in reducing anaemia (Wang et al., 2021); in Uganda, daily meals fortified with micronutrients reduced moderate-to-severe anemia in adolescent girls by 20 percentage points (Adelman et al., 2019). Interventions to improve water quality, hygiene behaviours and sanitation in schools can reduce absence among students (Freeman et al., 2012), and improved and gender-responsive WASH can reduce girls' absence from school during menstruation (UNESCO, 2014; Adukia, 2017; Kim and Rhee, 2019).

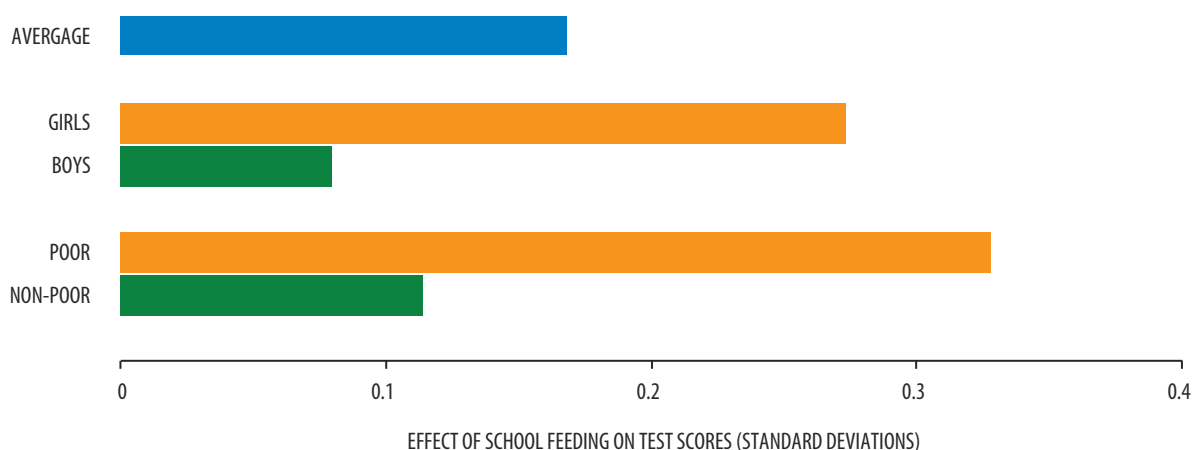
Providing school meals has been shown to increase enrolment, grade attainment and learning capabilities (Wall et al., 2022), and can particularly benefit girls and the most marginalized learners. In Ethiopia, students who received daily school meals were almost half as likely to miss class as those who did not and had test scores on average 2.4 percentage points higher than students who did not benefit from school meals (Desalegn et al., 2021). In Uganda, school feeding led to an increase in attendance of up to 15 per cent (Alderman et al., 2012). In Ghana, school meals increased test scores by 0.17 standard deviations on average, but the effect was almost twice as large among poor students (Figure 1) (Aurino et al., 2023).

Box 1. School health and nutrition programmes deliver big education outcomes

 Up to 2.5 years of additional schooling by providing school-based deworming and micronutrient supplementation where helminth infections and iron deficiency anaemia are prevalent (Bundy, 2011)	 21%–61% reduction in absenteeism in low-income countries by promoting handwashing (McMichael, 2019)
 20% reduction in moderate-to-severe anaemia in adolescent girls with daily meals fortified with micronutrients (Adelman et al., 2019)	 Reduced girls' absence during menstruation, by improving school WASH (UNESCO, 2014)
 Up to 0.5 years of additional learning when children with poor vision are provided with glasses (Glewwe, Park and Zhao, 2016)	 50% fewer school days skipped by tackling school violence and bullying. Students who are frequently bullied are twice as likely to skip school than those who are not (OECD countries) (UNESCO, 2019a)
 62% reduction in absenteeism by implementing malaria prevention interventions (Fernando, et al., 2006)	 Improved sexual and reproductive health by providing comprehensive sexuality education, including reducing the risk of HIV and rates of early and unintended pregnancy – a critical factor in school dropout among girls (UNESCO, 2019b)

Source: Adapted from UNESCO et al., 2023, p.37 (available under [CC BY-SA 3.0 IGO](#))

Figure 1. School meals in Ghana benefited girls and children from poor households the most



Note: Scores are based on tests in mathematics and literacy.

Data source: Aurino et al., 2023, p.95 (available under [CC BY 4.0](#))

2.2 Education improves learners' health and well-being

Education has positive long-term effects on health outcomes into adulthood, including improving overall health behaviours and lowering mortality (Cutler and Lleras-Muney, 2006; Zajacova and Lawrence, 2018; Raghupathi and Raghupathi, 2020). A number of different factors underlie the relationship between education and health, including economic and social pathways and differences in access to health care. Education – and gender equality within education – can lead to more gender-equal social norms and other development outcomes (Branisa et al., 2013; Kim, 2021; Tokal et al., 2023), which are in turn a major social determinant of health (Veas et al., 2021). Another factor is that staying in school for longer has a protective effect against some health risks. In Botswana, for example, each additional year of secondary schooling during the late 1990s and early 2000s led to a reduction of 8 percentage points in the risk of HIV infection, with larger effects experienced by women and girls (De Neve et al., 2015).

The extent to which education fosters better health and well-being depends on the quality of education and the skills provided to learners. Skills-based education on health and well-being, including nutrition education, physical education and education that builds non-cognitive skills (perseverance, empathy, teamwork etc.), is particularly important. Comprehensive sexuality education – a curriculum-based approach to age-appropriate teaching and learning about all aspects of sexuality (cognitive, emotional, physical and social) (UNESCO 2018) – has been shown to reduce the number of unintended pregnancies and rates of HIV and other sexually transmitted infections among adolescents (UNESCO, 2019b). Education on menstrual health and hygiene is important from age 9 onwards to give all students accurate information and to ensure that female students understand how to manage their periods. However, currently only 2 in 5 schools globally provide menstrual health education (UNICEF and WHO, 2024). Social and emotional skills are shown to correlate with students' sense of well-being and with their health-related behaviours (OECD, 2024). Non-cognitive abilities, such as perseverance and self-control, significantly influence multiple social and economic outcomes, including employment prospects, educational attainment, early pregnancy rates, health behaviours, crime and academic performance (Cunha and Heckman, 2010).

2.3 Investing in health and well-being in education is cost-effective and improves equity

Interventions supporting health and well-being in education can contribute not only to improving health, nutrition and education outcomes but also to enhancing social protection, local food systems and dietary preferences. Schools offer a cost-effective means for the equitable delivery of health and nutrition services and are particularly well-positioned to enhance the health and well-being of children and adolescents in countries with weak health systems (Bundy et al., 2018). Moreover, school-based mental health and psychosocial support not only improves mental health outcomes but also yields substantial economic benefits: for children and adolescents affected by emergencies, every dollar invested in social and emotional learning averts an estimated US\$225 in lifetime earning losses (UNICEF, 2023). Schools are well placed to address mental health challenges – which can otherwise go undiagnosed and untreated – by promoting well-being, teaching about mental health and identifying early signs of distress (UNICEF, 2023). Inequalities in health and well-being, often driven by poverty, food insecurity and gender inequality, further highlight the critical role of schools. By addressing these underlying factors, school health and well-being initiatives can significantly reduce inequalities in learning as well as in life outcomes (UNESCO et al., 2023).

Health and well-being in education should not be seen as a competing priority with foundational learning when it comes to political attention or funding. Instead, adopting a holistic approach to learners' needs – one that supports their health and well-being and ensures a healthy school environment – is integral to supporting foundational learning. As the Convention on the Rights of the Child states, education should be directed to the 'development of the child's personality, talents and mental and physical abilities to their fullest potential' (United Nations, 1989). Moreover, foundational learning is embedded within a broader vision of education articulated by the UN Sustainable Development Goals, which state that education is expected to equip learners with the knowledge and skills they need to promote sustainable development – including about health, human rights, gender equality and peace (UNESCO, 2015).

3. The scale of national school health and well-being programmes and policies

Recognizing the importance of health and well-being in education, many countries have already implemented policies and programmes in schools. Globally, 90 per cent of countries have some form of school health and nutrition programme. More than a hundred countries have school-based vaccination programmes. In low- and middle-income countries, over 450 million school-age children are dewormed every year in schools (UNESCO et al., 2023). Almost every country in the world includes some form of education for health and well-being in its curriculum (see Figure 3). Over 40 per cent of children in primary schools around the world benefit from school meals (WFP, 2023). Most countries have a policy framework for school health and nutrition, sometimes integrated within broader education and health policy frameworks (UNESCO et al., 2023), and many countries have some kind of mental health programme in place (WHO, 2021).

Education sector plans (ESPs) – plans that provide a long-term vision for national education systems and a coherent set of strategies to reach the stated objectives – are important entry points for improving health and well-being in education (UNESCO-IIEP and GPE, 2015). Over the last ten years, several countries have incorporated health and well-being goals into their ESPs (Box 2). In countries in sub-Saharan Africa, there was a notable rise in health and nutrition interventions included in ESPs from 2000 to 2015 (Sarr et al., 2017).

In addition, country commitments in international forums show increasing ambition in relation to health and well-being in education. In practice, however, concrete strategies are sometimes constrained by situational challenges and significant financing gaps that lead to varied coverage and implementation, resulting in efforts that often fall short of goals to reach all children with a holistic package.

Box 2. Examples of issues related to health and well-being in education sector plans

Egypt: The 2023–2027 ESP recognizes that there are constrained resources for school feeding and WASH implementation.

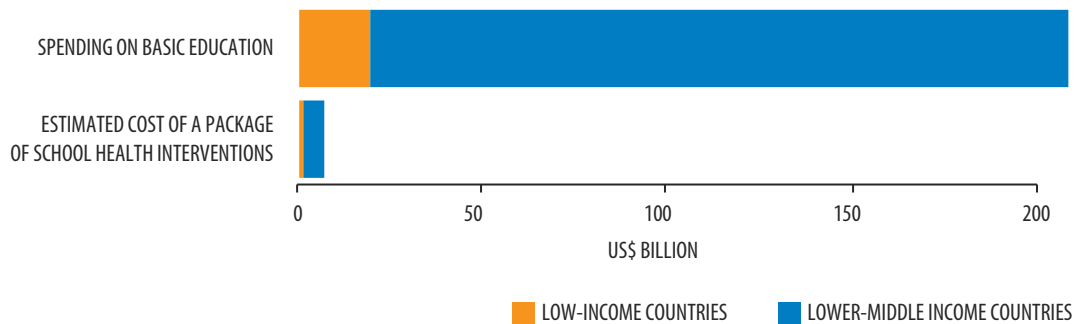
Nepal: The 2022/23–2031/32 ESP includes a provision for basic health and nutrition services, WASH and measures to address abuse, discrimination and bullying.

Liberia: The 2022/23–2026/27 ESP provides for skills-based health education through girls' clubs, expanding nutrition services and a focus on reducing gender-based violence in order to create healthy, safe and protective learning environments.

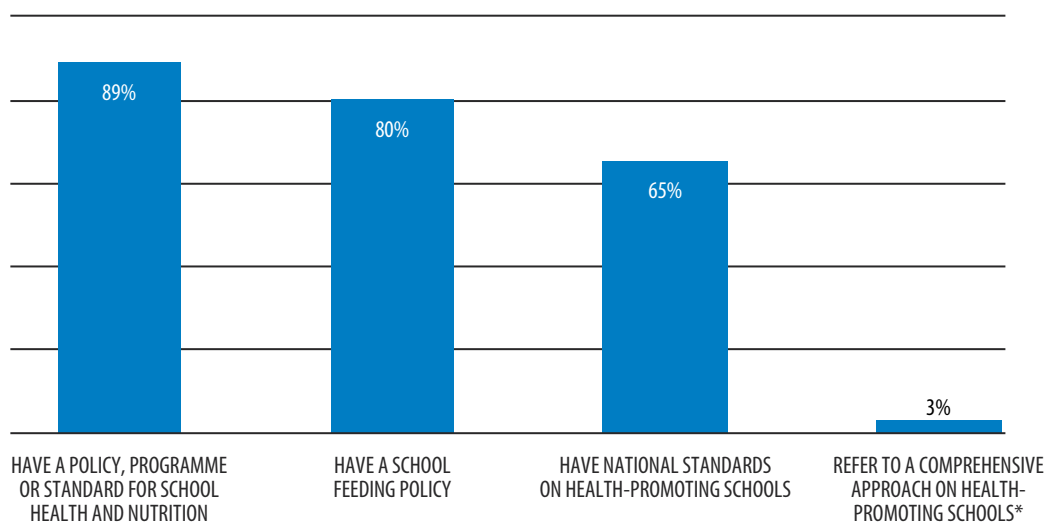
Source: Abduvahobov et al., 2024 (available under [CC BY 4.0](#))

As a result of these shortcomings, many children still do not benefit to the extent they could from health and well-being interventions in education, particularly at the post-primary level. Low- and lower-middle-income countries spend around US\$210 billion annually on basic education, but only US\$2 billion on the health of children aged 5–19. A package of school health investments costing around US\$7 billion per year could significantly optimize the benefits of education in low- and lower-middle-income countries (Figure 2) (Bundy et al., 2018).

The integration of health and well-being components into ESPs is uneven and fraught with challenges. Almost all countries around the world have some kind of policy, programme or standard related to school health and nutrition, and most have a national standard on health-promoting schools, but only 3 per cent of low- and middle-income countries are currently applying a comprehensive approach (Figure 3) (UNESCO et al., 2023).

Figure 2. Spending on basic education against the estimated cost of meaningful school health interventions in low- and lower-middle-income countries

Data source: Bundy et al., 2018, p.6 (available under [CC BY 3.0 IGO](#)). This is an adaptation of an original work by the World Bank. Views and opinions expressed in the adaptation are the sole responsibility of the author or authors of the adaptation and are not endorsed by the World Bank.

Figure 3. Percentage of countries that have plans and policies related to school health and well-being

*for low- and middle-income countries only

Data source: UNESCO et al., 2023 (available under [CC BY-SA 3.0 IGO](#))

For many countries attempting to address health and well-being in education, barriers to reaching the full potential of interventions and achieving national coverage include the following:

- Selection processes that do not always favour interventions with the strongest evidence of impact
- Small-scale implementation of interventions relative to the level of need
- Lack of coordination between ministries of education and other relevant authorities (e.g. unclear financing responsibilities for health, nutrition and WASH in schools)
- Lack of robust and comprehensive data on the health and well-being of children and adolescents (e.g. under-reporting of gender-based violence, lack of financial data on school feeding programmes and insufficient focus on children with disabilities)

- Weak monitoring and evaluation of outcomes
- Lack of institutionalized training for those implementing the interventions
- Insufficient funding
- Absence of standardized definitions of health policies, structures and interventions
- Gaps in strategic planning knowledge, skills and human resource capacity
- Low levels of stakeholder engagement in strategic planning processes

(Fathi et al., 2014; Shahraki-Sanavi et al., 2018; FAO, 2021; Bantilan et al., 2023; UNESCO et al., 2023).

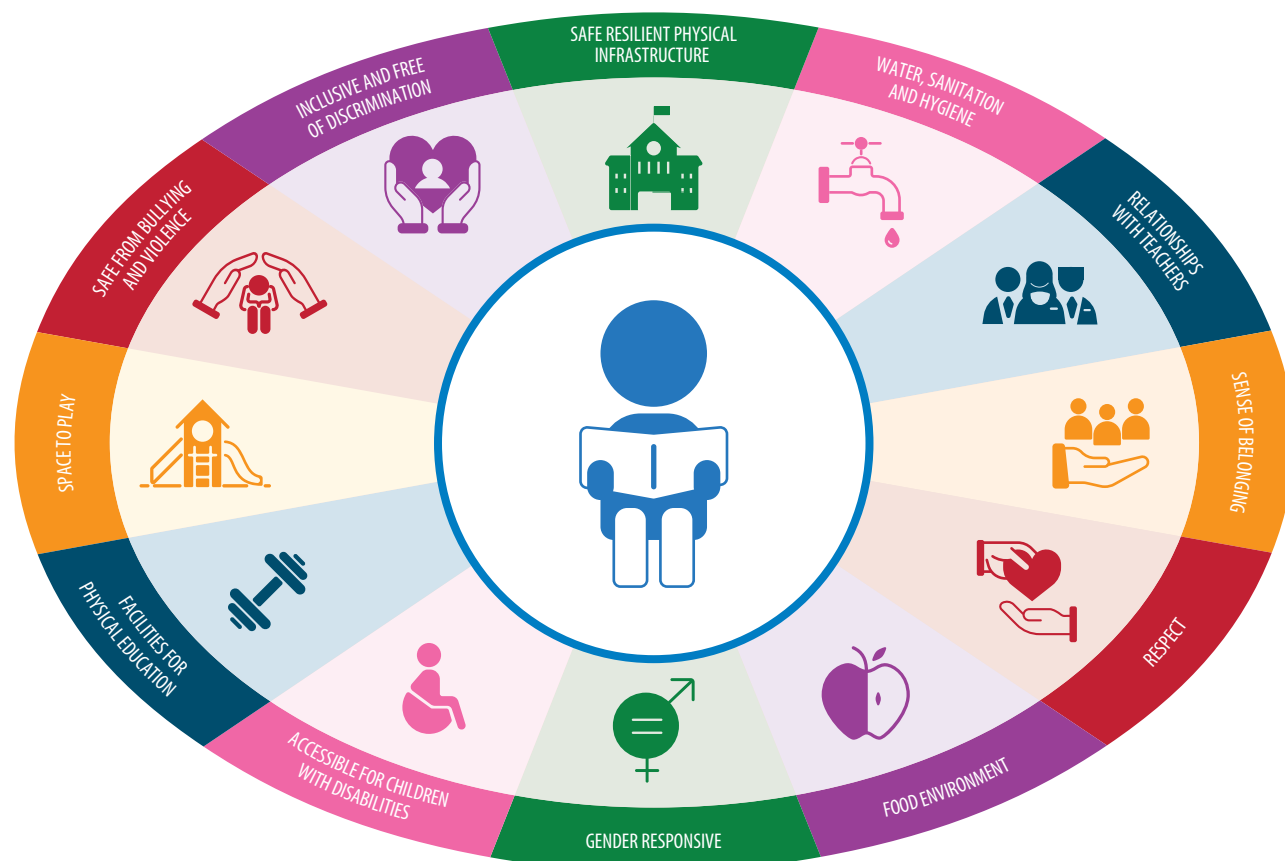
Overcoming these barriers requires strong cross-sector coordination (especially among education, health and other relevant ministries), institutionalized and participatory planning processes led by education authorities, and the use of robust data and evidence to ensure that education systems can effectively promote the health, nutrition, safety and well-being of all learners.

4. Creating enabling school environments for transformative learning

Despite the limitations and aforementioned barriers to implementation, many countries have made great progress in rolling out school health and nutrition programmes and services. However, there has not been enough attention given to other crucial components of health and well-being

in education, such as the school environment; skills-based education on health, nutrition and well-being; and more broadly, education that prepares learners for healthy, productive and sustainable futures.

Figure 4. Safe school environments that promote health and well-being

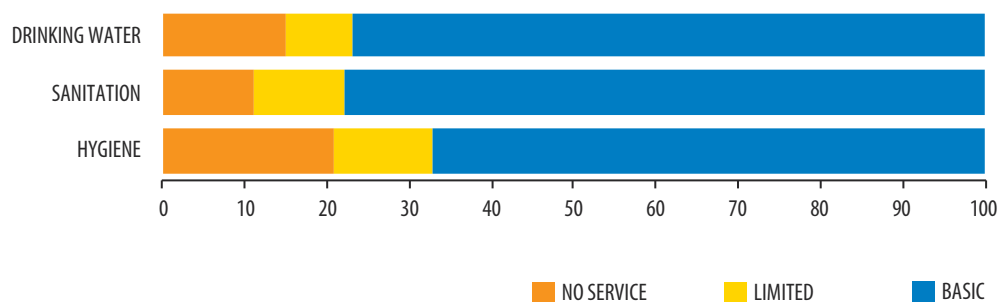


4.1 School environments

Students need safe, inclusive, health-promoting school environments in order to learn effectively. School environments that are conducive to learning are those that are free from bullying, discrimination or violence (Safe to Learn, 2023); those that promote gender equality; and those that support learners' mental health and social and emotional development. Safe school environments recognize the different risks that affect girls and boys and protect learners from sexual and gender-based violence; they have physical infrastructure that is safe for all and accessible to children with disabilities; and they have adequate spaces for play, facilities for physical education, drinking water, handwashing facilities and toilets that are gender-responsive and meet the needs of menstruators. A health-promoting school environment also encourages healthy eating and limits the marketing of unhealthy foods (WHO, 2023).

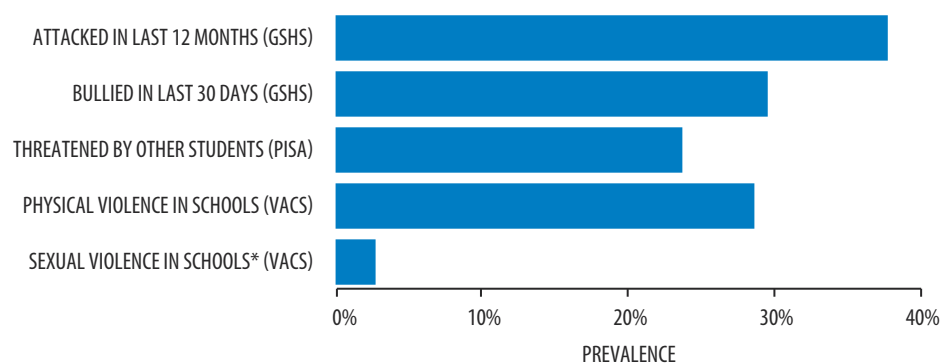
Despite the efforts of the past decades, more than 1 in 5 schools globally does not have safe drinking water and adequate sanitation (Figure 5). Bullying, corporal punishment, sexual and gender-based violence, and other forms of violence remain common in and around schools, with damaging effects on attendance and learning (Figure 6) (Evans and Hares, 2023). Many countries fall short of considering how the school environment as a whole can promote health and well-being (UNESCO et al., 2023). For example, an estimated 14 per cent of adolescents worldwide have a mental disorder (WHO, 2022), yet only half of countries provide school-based mental health programmes (UNICEF, 2021; WHO, 2021; UNESCO et al., 2023). Increasingly, learners in many countries spend time in digital learning environments, which also need to be safe, inclusive and health-promoting.

Figure 5. Drinking water, sanitation and hygiene in schools globally in 2023



Data source: UNICEF and WHO, 2024.

Figure 6. Prevalence of different forms of violence in schools as a percentage of students (according to self-reporting in multi-country surveys)



*may be underestimated

Data source: Wodon et al., 2021, p.2 (available under [CC BY 3.0 IGO](https://creativecommons.org/licenses/by/3.0/)). Data are collated from the following surveys, which each use different country sets: Global School-based Student Health Surveys (GSHS), Programme for International Student Assessment (PISA), and Violence Against Children Surveys (VACS)

4.2 Skills-based education on health and well-being

Health-related subjects are part of the curriculum in most countries (Figure 7), many of which have national standards on health education. However, even when health-related topics are included in school curricula, they are not always taught effectively due to ‘competing curriculum pressures, lack of quality learning resources and capacity development, and lack of teacher confidence in applying practical and learner-centred approaches’ (UNESCO et al., 2023). Health-related topics and subjects are likely to include sexual and reproductive health, including HIV and AIDS education, disease prevention, antimicrobial resistance, comprehensive sexuality education, menstrual health and hygiene education, mental health awareness, physical education and nutrition education. Curriculum content needs to respond to the real needs and experiences of learners based on health and behavioural data. In addition, learning approaches need to focus on building skills or changing behaviours, with adequate time in the curriculum for subjects and consistency across grades and school levels. Competencies in health and well-being can also be built beyond the school, as has been done, for example, in Lesotho, where a World Bank project funds youth clubs and safe spaces where adolescent girls and boys can develop life skills and awareness around sexual and reproductive health (World Bank, 2022).

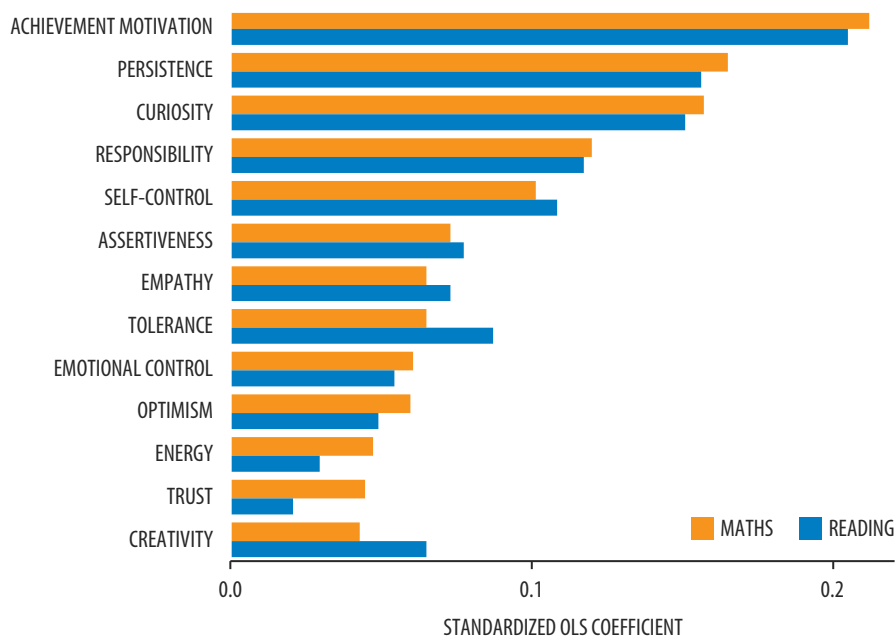
Figure 7. Education for health and well-being in the curriculum



Source: Adapted from UNESCO et al., 2023, p. 72 (available under [CC BY-SA 3.0 IGO](https://creativecommons.org/licenses/by-sa/3.0/)). With additional data on menstrual health education from UNICEF and WHO, 2024, p. 57.

4.3 Transformative education that prepares learners for their future

For education to realize its transformative potential, it needs not only to encompass health and well-being topics but also to equip learners with knowledge and skills in relation to gender equality, violence prevention, climate change, sustainable development, global citizenship and peace and to ensure that learners develop the socio-emotional skills they need not only to learn effectively but also to become healthy and productive adults. Many schools and school curricula do not make provisions to teach these critical skills to learners. Comprehensive policies on health and well-being in education recognize the connection between these elements and achieving truly transformative education. Non-cognitive skills play a fundamental role in life outcomes, including in health and education (Heckman, 2006; Cunha and Heckman, 2010). Social and emotional skills differ by family background and gender and are strongly associated with both well-being and learning outcomes in academic subjects (Figure 8) (OECD, 2024). Moreover, education needs to address the social norms and inequalities that determine societal health disparities. In doing so, education can become transformative, creating populations with the knowledge and skills to contribute to needed societal changes such as reducing violence, promoting gender equality and building inclusive and equitable institutions.

Figure 8. Social and emotional skills are associated with better learning

Data Source: Adapted from OECD, 2024, p.113 (available under [CC BY 4.0](#)). This chart is an adaptation of OECD data. The adaptation does not represent the views of the OECD or its Member countries.

Note: The standardized OLS coefficient represents the effect of a one standard deviation change in grades associated with a one standard deviation change in each skill (controlling for gender, socio-economic status, migrant status and school).

5. Integrating health and well-being into education systems

Pioneering approaches in countries such as Kenya and Indonesia highlight the effectiveness of a unified government strategy involving multiple line ministries – education, health, food and agriculture – working alongside departments responsible for social protection and regional administration to promote inclusive education through comprehensive school health and nutrition programmes (Bundy, 2011). In Scotland (United Kingdom of Great Britain and Northern Ireland), health and well-being have been integrated at the centre of education policy, with interventions ranging from mental health support to school meals and curricula designed to ensure that students acquire the skills they need to support healthier lives (Beadle et al., 2021).

Many countries, though, continue to struggle with adopting a holistic approach to health and well-being in education sector policy and planning. The focus often remains on the delivery of school health and

nutrition services without considering how the broader school environment and curricula can support learners' well-being, build life skills and promote resilience (Abdulahobov et al., 2024). The challenge often stems from a lack of clear guidance. While there are many frameworks, guidelines and analytical tools available that are vital in shaping how countries around the world prioritize and plan in the education sector (UNESCO-IIEP, World Bank and UNICEF, 2014; UNESCO-IIEP and GPE, 2015; UNGEI and GPE, 2017), their coverage of issues such as health, nutrition and well-being remains relatively limited. Many guidance documents primarily emphasize traditional education metrics, such as enrolment, efficiency, access, cost and financing. Education planners need to widen their focus to realize a more transformational vision of education – one that promotes equity, inclusion, health and well-being through participatory, interactive and learner-centred approaches.

More critically, there are a number of tools that have been developed since the early 2000s that have not yet been incorporated into the guidelines used by education planners. These tools are designed to help countries and education systems collect data, assess the status of their policies and practices, and identify areas for improvement in relation to health and well-being in education.

First among these tools are frameworks that can be used in education sector planning to ensure that analyses of health and well-being in education – including dialogue, consultation and planned policy and programme responses – are holistic and coherent, and that they reflect a suitable level of ambition in terms of education and health interventions that mutually support each other. These frameworks include the concept of health-promoting schools articulated by WHO, UNESCO and UNICEF in 1995, including a framework of global standards (WHO and UNESCO, 2021). Under the Systems Approach for Better Education Results (SABER) initiative, a multi-

agency team created a comprehensive framework that allows countries to analyse and strengthen aspects of their school health and nutrition policies and systems (World Bank, 2016). Other frameworks, standards and indicators are available on adolescent well-being (Ross et al., 2020), adolescent health (WHO, 2024), sexuality education (Hera et al., 2018; UNESCO, 2018), school food and nutrition (FAO, 2019), and violence against children, including school-related gender-based violence (UNESCO and UN Women, 2016; UNGEI, 2019).

Diagnostic tools – including many based on these frameworks – can guide policy-makers and planners through the process of analysing the components of health and well-being in education so that they can reach a better understanding of what the barriers are to implementation, what the strengths and weaknesses are of current policies and programmes, and what adaptations and new resources might be needed to speed up progress (Table 1).

Table 1. Policy diagnostic tools for health and well-being in education

Tool	How it can be used
<u>Systems Approach for Better Education Results (SABER) School Health and School Feeding tools</u> . A forthcoming Healthy-SABER will combine elements of these two tools (Schultz et al., 2024).	SABER serves as a guiding framework for systematically collecting information about the quality of school meals and school health policies. It helps benchmark current policies against good practice and identifies actionable priorities. An updated Healthy-SABER tool will support consensus-building on realistic national commitments to strengthen school health and school meals programmes.
<u>Assessment of National Education Systems for Health (ANESH) 2023</u> (ANESH, 2023)	ANESH assesses national policy frameworks for school health by examining coordination mechanisms, monitoring and evaluation systems, curricula and school health services.
<u>Sexuality Education Review and Assessment Tool (SERAT)</u> (UNESCO, 2020)	SERAT collects data on school-based sexuality education programmes through key informant interviews and desk reviews. It generates reflections and discussions about the strengths and areas for improvement in country contexts, covering national policy, curriculum content and programme characteristics.
<u>Comprehensive School Safety Policy Survey</u> (GADRRRES, n.d.)	This survey collects data on various risks and impacts affecting the education sector, including disasters, climate change impacts, conflict, bullying and violence, and assesses the extent to which these risks are addressed in policy and legal frameworks.
<u>Safe to Learn diagnostic tool</u> (Safe to Learn, 2021)	This diagnostic tool measures the quality of national efforts to prevent and respond to violence in and around schools by assessing checkpoints at national, subnational and school levels.
<u>RAMES: Review and Assessment of MHPSS in Education Systems</u> (UNESCO, 2024a)	RAMES reviews and assesses the readiness of education systems to provide mental health and psychosocial support (MHPSS). This tool includes desk reviews, stakeholder consultations and a final assessment report to highlight areas for improvement.

The combined voices of students and teachers is a readily available resource in all countries. These important stakeholders have first-hand knowledge of health-promoting and health-limiting factors within their schools and can contribute ideas for improvement, informed by their day-to-day experiences and real-life context.

As policy-makers and planners consider new areas of programming, they can draw on an increasing body of research and evidence reviews (Box 3) that consolidate the best available global insights and data on health and well-being in education. They can also draw on the data collected by applying survey instruments on child and adolescent health and school environments, such as the Global School-based Student Health Survey and the Global School Health Policies and Practices Survey, in addition to global data platforms that include indicators summarized in the 2023 report *Ready to Learn and Thrive: School Health and Nutrition around the World* (UNESCO et al., 2023).

Box 3. Evidence reviews about school health and nutrition

Evidence reviews consolidating the best available global evidence on interventions in relation to health and well-being in education:

Systematic reviews show the impact of school-based health and nutrition interventions on improved access to learning (Snilstveit et al., 2015; GEEAP, 2023).

Cost-effectiveness studies show that comprehensive school health and nutrition programmes provide the highest return on investment, making them the most economic approach to promoting long-term health outcomes (Ekwaru et al., 2021).

Other systematic reviews focus on specific types of intervention, such as providing school meals (Wang et al., 2021; Wall et al., 2022).

The third edition of the Disease Control Priorities series presents evidence on optimizing learning outcomes through interventions in school health and proposes a package of high-return investments (Bundy et al., 2018); the fourth edition contains updates on the evidence (Schultz et al., 2025).

The *INSPIRE Handbook* developed by WHO proposes seven strategies for ending violence against children, including in education, and documents the evidence and country case for each strategy (WHO, 2018).

These resources represent a growing body of work that can fill important gaps in guidance in education sector planning and help policy-makers and their partners incorporate evidence-based health and well-being interventions into education sector planning. However, it is important to recognize that within this goal, barriers remain to fully utilizing these tools in mainstream education planning processes. Overcoming these barriers will involve the following:

- A more complete understanding of the concepts and terminology around health and well-being in education
- Greater awareness of the tools that can support health and well-being in the education sector
- The practical capacity to apply these tools in education policy and programme planning
- Increased cross-sectoral expertise in health and well-being in education at subnational, national and international levels
- Competent coordination mechanisms and governance arrangements geared towards different ministries and organizations being able to address the health and well-being of learners in a complementary way
- Increased funding for action on health and well-being in education and the political will to mobilize funding

Countries may need greater external technical and financial support in order to overcome the barriers that exist. The tools discussed here are best used as a guide for policy-makers and planners and as a means to achieve a more holistic vision of transformative education that empowers learners to build fairer, more inclusive and more peaceful societies and provides them with the means to address the social determinants that lead to health disparities. This framing of what education can do brings together diverse elements of education planning and policy and helps planners to build these elements into a cohesive strategy.

6. Ways forward

Health and well-being, as well as being important goals in their own right, are essential factors in ensuring that all children and adolescents can achieve foundational learning outcomes and beyond. Commitments made at the 2022 Transforming Education Summit, including the Commitment to Action on Foundational Learning, recognize this potential. This initiative commits to supporting the health, nutrition and psychosocial well-being of every teacher and child and highlights the importance of foundational learning as a contributor to productive citizenship, sustainable development, gender equality, peace and other national and international goals (UNICEF, 2022; World Bank, 2022a).

Policy-makers, planners, international organizations, donors and civil society actors now need to take the next steps towards making this intersectoral work a reality. Education stakeholders and planners must develop coherent, comprehensive, evidence-based and context-specific packages of interventions. In doing so, education authorities should take the lead in mobilizing and convening other sectors, such as health, social protection and agriculture, through participatory processes that clearly define roles, responsibilities and the financial contributions of each sector. This collaborative approach will help establish a shared accountability and sustainable commitment towards improving health and well-being outcomes for all learners.

Moreover, to ensure that these efforts are strategic and effective, it is essential to leverage the latest evidence-based guidelines and diagnostic tools. These resources provide clear frameworks for defining the interplay between education, health and well-being, enabling governments to analyse, plan, finance, implement, monitor and evaluate policies and programmes with precision. Development partners, therefore, need to step up technical support so that governments can make full use of these resources.

This integration requires a shift towards more intersectoral approaches aiming to cultivate resilient, informed, healthy, thriving and global citizens who can contribute to the changes needed to make societies healthier. It means clearly defining the responsibility of each sector and level of administration involved in this collaborative effort and ensuring a mutual accountability towards progress. While guidance on the many aspects of health and well-being in education exists, this guidance has not routinely been part of the standard frameworks and toolkits that developing countries use for their education sector planning (Abduvahobov et al., 2024).

A core aspect of this transformative vision is for education sector stakeholders and planners to drive collaboration with other sectors, drawing in expertise on gender equality, health, social protection, community, food and agriculture, as well as ensuring that there are adequate funds to support child and adolescent development. The need for deeper cross-sectoral coordination and engagement in education sector planning has long been recognized, particularly for early childhood development programming (Vegas and Santibáñez, 2010). Yet this level of coordination remains an aspiration in many cases.

Technical support and tools do not by themselves solve all the issues facing education planning processes: a supportive political economy, capacity development and financing are needed to sustain developments in health and well-being in education. Similarly, global evidence production and synthesis is not in itself sufficient to ensure the use of evidence-based research at country level: locally produced evidence and learnings that are relevant to country needs and priorities are needed, including partnerships between evidence users and producers (Steiner-Khamsi and Faul, 2024). But showing that there is already a strong global evidence base for health and well-being in education and coherent, integrated technical guidance on how these aspirations can be put into practice is an essential foundation for mobilizing political and financial resources and for putting evidence-based policies and programmes in place.

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Transforming education through health and well-being

A briefing note

Healthy, safe and inclusive learning environments are a foundation for quality education. This briefing note makes the case for integrating health and well-being into education policy and planning so that all learners can learn, thrive and reach their full potential.

Drawing on global evidence, tools and country experience, it highlights how investing in school health and well-being improves learning outcomes, promotes equity and strengthens resilience. A companion to UNESCO's handbook on integrating health and well-being in education policy and planning, the briefing note demonstrates the place of health and well-being in a wider, holistic vision to transform education.

